

Texas WIC Health and Human Services Commission

Effective April 1, 2026

Policy: CR:07.0(eWIC)

Provision of Services to Families with Special Health Care Needs

Purpose

To eliminate barriers to WIC services for families with special health care needs (FSHCN) and ensure nutrition education bears a practical relationship to the nutritional needs of the participant.

Authority

7 CRF Part 246.8, 246.11

Policy

Each local agency (LA) must make accommodations and provide services for FSHCN.

Definitions

Electronic Benefit Account – The account where food benefits are being held for redemption.

Families with special health care needs (FSHCN) – have a family member who is a(n):

1. Infant or child on the WIC program, who has:
 - A health condition that has a biologic, psychological, and/or cognitive basis which has lasted or is virtually certain to last for at least a year; or
 - A condition(s) which results in limitation of function, activities, or social roles in comparison with healthy age peers in the general areas of physical, cognitive, emotional, and social growth and development; or

Texas WIC

Health and Human Services Commission

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- A need for medical care and related services, psychological services, or educational services over and above the usual for the child's age.
- 2. Pregnant, breastfeeding or postpartum WIC participant with an acute or chronic medical condition that affects her ability to participate in WIC activities; or
- 3. Primary caretaker with a special health care need: this person is not a WIC participant but is a parent or guardian of someone enrolled in WIC. Due to their medical condition this person is unable to participate in WIC program activities; or
- 4. Another family member that is not a WIC participant with special health care needs who prevents the participant/parent/guardian/caretaker from participating in WIC activities. For example, a family member with Alzheimer's disease who needs constant supervision and should not be left unattended while a mother takes her child to the WIC clinic.

Individuals with special health care needs – have conditions referred to as medically complex, medically fragile, or severely disabled.

Medically Complex – Individuals with a serious, ongoing illness or a chronic condition that has lasted or is anticipated to last at least twelve or more months and require daily, ongoing medical treatments and monitoring by appropriately trained personnel (which may include parents or other family members) and require access to a complex array of services.

Medically Fragile – Individuals who have a serious, ongoing illness or a chronic condition that has lasted or is anticipated to last at least twelve or more months or has required at least one month of hospitalization, and requires daily, ongoing medical treatments and monitoring by appropriately trained personnel (which may include parents or other family members). Their care requires the routine use of a medical device or assistive technology to compensate for the loss of usefulness of a body function

Texas WIC Health and Human Services Commission

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Policy: CR:07.0(eWIC)

needed to participate in activities of daily living. These individuals live with an ongoing threat to their continued well-being.

Severe disabilities with special health care needs – A person with severe disabilities requires or has the potential to require the individual and/or their family/caretaker (in relationship to the individual's care) to access a complex array of services from public/private service providers in order to achieve optimal care for the individual. The access/coordination of these services may exceed the immediate capabilities of the family/caretaker. The services are anticipated to be lifelong or of extended duration and require individualized planning and coordination.

WIC Card Management Line – An interactive voice response system where clients can manage the Texas WIC Card (set/reset the PIN, cancel/replace the card), obtain food benefit balances, and speak to a customer service associate.

Procedures

- I. When FSHCN are identified, the family must be considered as a unit and accommodations made accordingly. To determine the length of the physical presence waiver and documentation of physical presence, refer to [CS:04.0 Physical Presence](#).
- II. When a parent/guardian/caretaker or participant is unable to be physically present, another adult may be authorized to provide the required certification documents, be the food benefit, recipient, and for initial certification accept the Texas WIC Card. Staff must inform the authorized adult the primary endorser must set the PIN for the Texas WIC Card via myWIC or WIC Card Management Line. For a sub-certification, the authorized adult must provide the following:
 - A. A signed note from the parent/guardian/caretaker stating they are attending the WIC appointment in place of the parent/guardian. The authorized adult must provide all documents and information required for income screening and nutrition assessment (spouses

Texas WIC Health and Human Services Commission

Effective April 1, 2026

Policy: CR:07.0(eWIC)

do not need to bring a note), if the primary endorser has not provided/uploaded documents.

- B. When the authorized adult brings note and documents to the appointment, staff must upload all documents into the MIS. Any documents obtained via electronic transmission will automatically be uploaded and saved in the MIS.

III. FSHCN must be offered special accommodations to include:

- A. Obtaining medical information from participant's health care provider (HCP):
 - 1. Medical information including blood iron levels, length/height, weight, medical history, and/or dietary prescription can be provided by the HCP either verbally or in writing (e.g., fax, email) for certification purposes. (Refer to [CS:17.0 Documentation of a Complete Nutrition Assessment](#).)
 - 2. When Medical Information is not available at the time of certification:
 - a. Defer the anthropometric and blood iron levels for up to 60 days according to Policy CS:04 Physical Presence.
 - b. For dietary prescription and medical history information, follow guidelines in Policy FD:16.0 Approval of Non-Contract Formula
 - c. When medical information is waived, in the MIS staff must select "weight not available" and/or "height not available" as needed in the anthropometric record and select the reason for waiving the blood iron level in the blood work record.
 - 3. When Medical Information is not available and cannot be obtained due to the participant's medical condition (e.g., contractures, medically complex or fragile and/or severe

Texas WIC Health and Human Services Commission

Effective April 1, 2026

Policy: CR:07.0(eWIC)

disability), written verification must be obtained from the HCP indicating that the participant is under their ongoing medical care and uploaded to the MIS. Medical information is waived for the certification period.

- B. Offering coordination with social service agencies to provide transportation to, and shopping assistance at WIC vendors.
- C. Participants who receive nutrition counseling from an outside source meet the nutrition education requirement and do not need to come to the clinic for benefit issuance. Food benefits will be assigned to the economic unit's EBA.
- D. Providing relevant nutrition education (NE) by the most appropriate method(s), as determined by the CA:
 - 1. Give the participant/guardian/caretaker the option of attending an NE class, completing an online lesson or a self-paced lesson. If the participant/guardian/caretaker refuses NE, select "Attended – Refused NE" from the drop-down list from the scheduled class in the MIS.
 - 2. Provide individual counseling by a Registered Dietitian (RD) knowledgeable in the area of special needs. Counseling may be provided in the office, by telephone, remote visit, home visits, or hospital visits. LAs are encouraged to be flexible in scheduling services to meet the family needs (i.e., scheduling appointments during off-peak hours when feasible).
 - 3. Substitute nutrition education for counseling provided by a qualified outside source (i.e., Registered Dietitian (RD), Licensed Dietitian (LD), or gastroenterologist to serve as a nutrition education contact (refer to [NE:01.0 Standards for Nutrition Education Contacts](#)). Document this in the MIS.
 - 4. Refer the participant for individual counseling to a qualified outside source.

Texas WIC Health and Human Services Commission

Effective April 1, 2026

Policy: CR:07.0(eWIC)

- E. Referring to appropriate health and social service programs for which the participant may be eligible (e.g., Medicaid, ECI, SSI, etc.).

IV. Required Documentation in the Participant Page of MIS (A – D):

- A. Medical information taken verbally from the child's healthcare provider.
- B. The special health care needs of the family verbally provided by the participant. A statement from the physician is not required.
- C. Special accommodations/referrals that have been made and/or offered.
- D. Name and credentials of the qualified outside source that is providing nutrition counseling, if applicable. This information may be taken verbally from the participant.